National Cancer Control Programme (NCCP) 00432 · clinician draft

AC → weekly paclitaxel + weekly trastuzumab

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

AC · 21-day schedule

Active medicines: Doxorubicin + Cyclophosphamide

Emetogenic risk: High · AC

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 No routine Days 2–4 dexamethasone in this National Cancer Control Programme (NCCP) AC antiemetic row
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, interactions and glucose
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications

Additional phase-specific support

- Assess granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to source guidance and individual risk.
- This AC antiemetic script is replaced at the taxane transition.
- At the AC-to-paclitaxel transition, stop using this AC antiemetic template and select the taxane phase. Review residual symptoms separately.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00432 v5 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-23 · SACTCheck v0.84.17 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Taxane / combination phase

Active medicines: Paclitaxel + Trastuzumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (National Cancer Control Programme (NCCP) suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.
- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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HER2-only continuation

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Routine prophylactic antiemetics and paclitaxel premedicines are not carried into this phase. Treat prior nausea / infusion reactions under an individual plan.
- Continue HER2 cardiac monitoring; ending paclitaxel does not end echo surveillance.
- National Cancer Control Programme (NCCP) 00432 transitions after 12 weekly combination doses to trastuzumab 6 mg/kg every 21 days under regimen 00200, to complete one total year. Confirm the prescribed schedule and missed-dose rules; do not reuse weekly loading logic for 21-day continuation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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